

Caring for Autistic People of Every Age in Your Clinic

The cumulative guide for family doctors & health professionals — across the whole autistic lifespan, from pre-school to high-support older age. An extended reference sheet; pair it with the single-age quick guides.

The one idea

Autism, through **monotropism**, means attention locks into a few deep, intense “tunnels.” Inside a tunnel, focus is vivid; outside it, things barely register — sometimes including pain or someone speaking; and switching tunnels costs real energy. This single difference explains the sensory reactions, the strong interests, the need for routine, and the meltdowns/shutdowns that bring autistic people to your door. **Work with the tunnel, not against it.** Clinic is one of the hardest environments a monotropic person enters — small, predictable adjustments change outcomes.

What helps at every age

- **Send a pre-visit accommodations form** (the AASPIRE *Autism Healthcare Accommodations Tool*, autismandhealth.org) and read it before they arrive.
- **Lower sensory load:** dimmer light, quieter room, fewer beeps, no strong perfume; offer the first/last (quietest) slot.
- **Communicate literally and in writing:** explain each step *before* doing it; say what you mean; offer a written summary and written/online follow-up; allow long silences for processing.
- **Allow a trusted supporter and a sensory kit** (headphones, sunglasses, fidget/interest object); let the person look away, stim, or use AAC.
- **Co-create a predictable care routine:** same room, same sequence, effective adjustments saved to the record so they are automatic next time.
- **Treat the person as the expert on their own experience** — self-report is primary evidence.

What to avoid at every age

- Forcing eye contact, surprise procedures, restraint, or “be brave!” phrasing.
- Reading a calm, masked few minutes as “they’re fine,” or a shutdown as refusal to engage.

- Equating “not complaining” with “not suffering” (interoception differences mean pain and symptoms may be delayed or atypical).

Quick notes by life stage

Pre-school (2-5)

Slowed “sticky” attention, intense focused interests, sensory differences and atypical play are early signs — **refer early** for assessment. Join the child’s interest before examining; welcome a comfort object. Take parental concern seriously.

Primary school (6-11)

“Fine at school, collapse at home/clinic” is the classic masked pattern. Ask about **school load and recovery**, not just symptoms. Watch for burnout/“regression” under overwhelm — reduce demands, don’t push. Screen the common co-occurrences (anxiety, ADHD/AuDHD, sleep, GI).

Adolescence (12-18)

The **peak age for anxiety, depression and burnout**; many are first identified in crisis. **Screen for suicidality directly**. Distinguish autistic burnout (needs reduced demands + unmasking) from depression. Validate interests as identity and recovery. Plan the transition to adult services early.

University / college (18-25)

A massive transition; many hit a wall in year 1-2. Offer **written-first** options. Recognise burnout vs ordinary student stress. A temporary reduction in academic load is a legitimate recovery step, not failure.

Working adult

In the right environment: deep strengths. In the wrong one: chronic depletion, masking, **burnout** (distinct from occupational burnout and depression). Ask about workplace load; **start medications low and titrate slowly** (atypical responses are common). Signpost workplace accommodations.

Parent

Parenting is relentlessly high-channel — high **burnout** risk, and they often ignore their own health (interoception + hyper-focus on the child). Make perinatal care predictable and calm; proactively screen the parent’s own needs; they will under-report.

Older adult (60+)

The least-researched, worst-outcome group; many never identified. Watch for decades of accumulated burnout misread as “ageing.” **Distinguish autistic difference from dementia**

carefully — the evidence is thin; assess, don't assume. Protect lifelong interests and routines; account for hearing/vision loss compounding sensory differences.

High-support / residential care (any age)

Treat new distress as **pain or overload until proven otherwise** (constipation, dental pain, infection, ill-fitting equipment are common hidden causes). Use supported decision-making; preserve routines, objects and interests; have a calm meltdown/shutdown plan (reduce stimulation, ensure safety, no restraint).

Red flags that apply across the lifespan

Autistic burnout is not depression — chronic exhaustion, loss of skills and reduced tolerance to stimulus; treating it as depression or “try harder” deepens it and raises suicide risk. **Masking in clinic is costly** — don't let a composed 15 minutes override history. **“Not complaining” ≠ “not suffering”** — proactively ask about pain and symptoms. **New distress in a high-support person is a medical red flag** until pain and overload are excluded. Always centre the individual; a minority of autistic people don't identify with monotropism.

About this guide

*AI-assisted, derived from this project's research drafts — the **Lifespan Practitioner & Health-Care guide** (Parts 2–5) and the **Monotropism** brief. Uses identity-first language (“autistic person”). **Informational — not medical advice or a diagnostic tool.** Monotropism is a powerful but still-developing theory; old-age and high-support evidence is thin — be honest about uncertainty and fit it to the individual.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2018/2023); Dwyer (2021, 2025); Raymaker et al. (2020, burnout); Mantzalas et al. (2022); Pearson & Rose (2021, masking); Kelly Mahler (2024, interoception); Klein et al. (2024, aging); AASPIRE Healthcare Toolkit / AHAT; Autism Society (2025, meltdowns/shutdowns). Full citations are in the source drafts.